WRITING SAMPLE · POLICY ANALYSIS & PROGRAM EVALUATION

# Comparative Analysis of "High Acuity" in State Child Welfare Agencies

A fifty-one jurisdiction scan of how the term is defined, assessed and operationalized, based on direct agency outreach and documentary review.


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## Introduction

As part of ongoing efforts to enhance understanding of child welfare practices nationwide, this memorandum presents a comparative analysis of the term “high acuity” as interpreted and applied by state child welfare agencies across the United States.

In child welfare, the term “high acuity” carries significant implications for the care and services provided to vulnerable children and youth. Understanding the nuances of this term and its operationalization within different jurisdictions is crucial for developing effective strategies to address the complex needs of children and families involved with child welfare systems.

The purpose of this analysis is to provide a comprehensive overview of how “high acuity” is defined, assessed and addressed at the child and case level within state child welfare agencies. By examining the diverse approaches taken by different states, the analysis identifies common trends, disparities and best practices in addressing the needs of children with high acuity in the child welfare context.

## Methods

Appropriate staff at each state agency were surveyed by email for their agency's definition of high acuity in a child welfare context. Where a state did not respond, the term “high acuity” was searched on that state's child welfare agency web pages. Results were organized into three categories:

- **No definition** — the state did not apply the term in any observed context.
- **Public / mental health** — the state applied the term in public health and/or psychiatric contexts.
- **Child welfare** — the state applied the term in a child welfare context, non-exclusively.

Individual respondents at partner state agencies participated on the understanding that their responses would inform internal analysis. Their names and titles have been withheld from this version of the memorandum; responses are attributed to the responding agency and the month of response.

## Results

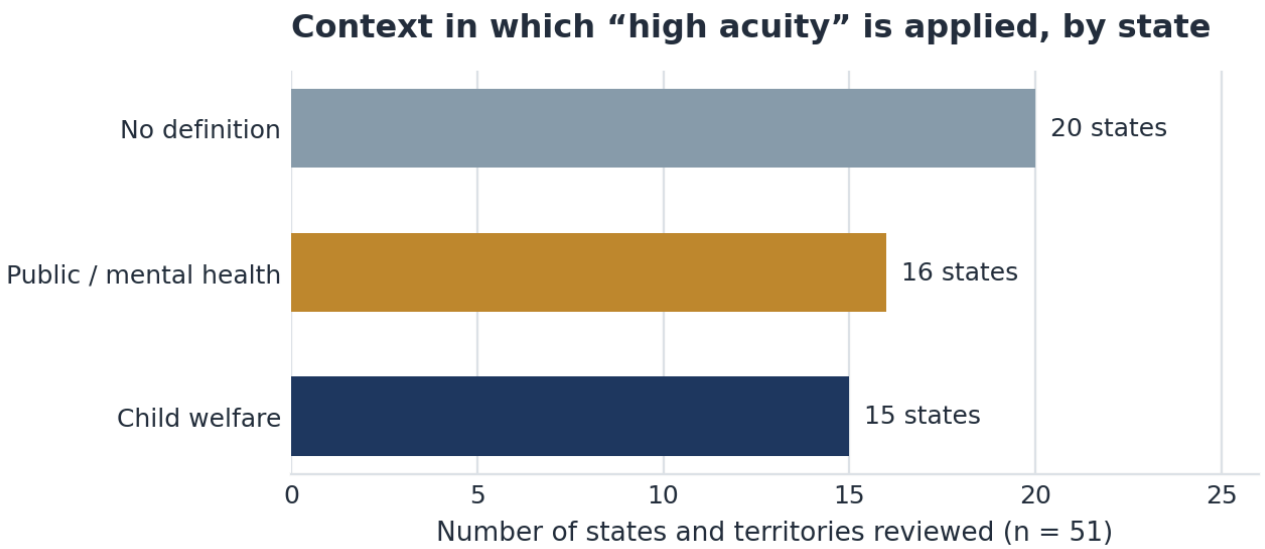



**Figure 1.** Context in which the term “high acuity” is applied, across the 50 states and the District of Columbia. Twenty states did not apply the term in any observed context; 16 applied it in a public or mental health context only; 15 applied it in a child welfare context.

## Best practices

In the context of state-level policies and practices, it is evident that the term “high acuity” is variably interpreted and operationalized. States exhibit divergent approaches when classifying children with complex needs or those requiring intensive care. These variations manifest not only in the use of the term itself but also in the adoption of distinct rating frameworks to assess the acuity level of such children. The nuanced interplay between terminology and assessment methodology underscores the complexity inherent in addressing high-acuity pediatric cases within the purview of state policies and services.

### California

In California, “high acuity” refers to the severity of a patient's illness or medical condition, indicating a state of severity and imminent danger. Patients classified as high acuity require urgent and substantial treatment to safeguard their health and safety, as well as that of others. Within emergency services specifically, high-acuity patients exhibit symptoms of life-threatening illness or injury necessitating immediate intervention to prevent further instability in vital functions such as airway, respiratory, hemodynamic or neurologic functions.

Although the term is referenced in certain documents and discussions, an explicit definition has not been identified. Instead, California employs the term “complex care” to delineate needs across multiple systems, and has formulated a definition accordingly.

Complex care delineates circumstances where youth exhibit a range of identified needs across multiple domains that have remained unaddressed, often over extended periods, by behavioral health, child welfare, probation, education and other systems. These youth consequently require high-intensity, individualized treatment options to effectively resolve challenges and meet their needs.[1]

### Kentucky

In Kentucky, the concept of “high acuity” refers to youth with distinctive, intense and co-occurring emotional, behavioral and occasionally physical care needs that standard systems have chronically failed to address adequately. Identifying such youth for case, program or systems-level oversight and evaluation proves challenging. However, certain indicators are used, including experiences of nontraditional placements, placement instability, hospitalization for inpatient psychiatric stabilization without an identified step-down placement, and completion of residential treatment programs without an identified step-down placement. To address these challenges, Kentucky has implemented various strategies, such as establishing specialist positions in each service region, forming interagency workgroups, and facilitating daily interagency meetings to discuss youth in nontraditional placements or expected to be placed nontraditionally.

Partnerships with behavioral health services aim to increase access to waiver services, mobile crisis response and youth peer support. Creative partnerships with service providers ensure appropriate placement and treatment options, considering the unique needs of high-acuity youth. Identified trends among this population include involvement with the justice system, experiences of trauma, familial substance misuse, adoption disruption, and survivors of human trafficking. These efforts aim to address the complex needs of high-acuity youth, predominantly teenagers, who often feel hopeless and isolated.[2]

### Missouri

In Missouri, the term “high acuity” is not applied in the context of child welfare, though it is used in medical settings. Instead, the state operates a program known as “Elevated Needs.” This program is intended for foster youth who have moderate to severe emotional and/or behavioral issues that are identifiable and documented. These youth require intensive, personalized interventions to thrive in a family setting within the community and to reach their goal of a permanent home. The program offers two levels, Level A and Level B, to cater to the specific needs of the child.[3]

### Oregon

In Oregon, the term “high acuity” is not explicitly defined in rules related to children. Instead, the state's systems of care typically use terms such as “complex,” “specialized,” or “extraordinary needs” for children who are considered highly acute. These needs

primarily encompass children with active psychiatric instability, intensive behavioral health needs, active homicidal or suicidal ideation, active sexual offending behaviors, or those who are actively physically aggressive. Children with intellectual and/or developmental disabilities in combination with any of these needs are also included.

Oregon defines “intensive treatment services” in Oregon Administrative Rule 309-022-0105(46). These include services provided by psychiatric residential treatment facilities and psychiatric day treatment services, or other services as determined by the Division, aimed at providing active psychiatric treatment for children with severe emotional disorders and their families.

Oregon also uses a standard intensive outpatient care model known as intensive in-home behavioral health treatment. Eligibility for this service, as outlined in Oregon Administrative Rules 309-019-0167, 410-172-0650 and 410-172-0695, requires children under 21 years to exhibit intensive behavioral health needs, including multiple behavioral health diagnoses, impact on multiple life domains (school, home, community), significant safety risks, risk of out-of-home treatment or placement, or transitioning from out-of-home treatment or placement.[4]

### Rhode Island

Rhode Island does not have a concrete definition for “high acuity.” However, the state uses a version of the Child and Adolescent Needs and Strengths tool. This tool, combined with the state's own algorithm, classifies child acuity into five tiers. Tier one represents the least acute cases, while tier five represents the most acute. These tier scores are instrumental in determining when a child requires more intensive support, with children in tiers four and five requiring the highest level of support.[5]

### Utah

In Utah, “high acuity” refers to individuals who have multiple complex needs and pose a risk to the public, making it challenging for a single department to provide adequate services. These individuals require a diverse and adaptable range of services and supports. Many young people who meet this criterion become among the most expensive cases, due to frequent interactions with psychiatric inpatient care, expulsions from residential programs, and common appearances in emergency departments.[6]

### Virginia

In Virginia, “high-acuity youth” is defined at the child level as young individuals under the care of local social services departments. These individuals are involved in multiple systems, such as child welfare, juvenile justice and mental health, and have complex medical and/or behavioral health needs that typically surpass the capabilities and resources of any single provider or system.

In practical terms, particularly in the context of Virginia's Safe and Sound Task Force, “high acuity youth” refers to young individuals with a range of complex, high-level needs who are either currently displaced or at immediate risk of displacement.[7]

### Washington, D.C.

Washington, D.C. uses the Child and Adolescent Functional Assessment Scale to assess a child's day-to-day functioning every three months across critical life subscales, to determine whether functioning improves over time. The result of this score determines the acuity score for the child: low acuity is 70 and below, high acuity is 80 to 130, and severe acuity is any score above 140.[8]

### Washington

In Washington, the term “high acuity” is not commonly used within the state's terminology. Instead, the focus is on addressing the needs of youth requiring a higher level of care and supervision, often referred to as “intensive resources.” This term is employed in efforts to serve this particular population. At the case level, however, there is no specific term designated to describe high acuity. Practitioners are aware of cases with high intensity or high needs, but systematic identification, evaluation or targeting of such cases may not occur. Alternatively, there may be a focus on high-profile cases or those with critical incidents, which are tracked and defined.[9]

# Appendix — Classification by state

**Table 1.** Context in which "high acuity" is applied, by state and the District of Columbia.

| State | Context | State | Context |
|---|---|---|---|
| Alabama | No Definition | Nebraska | Public/Mental Health |
| Alaska | Public/Mental Health | Nevada | Public/Mental Health |
| Arizona | No Definition | New Hampshire | Public/Mental Health |
| Arkansas | Public/Mental Health | New Jersey | Public/Mental Health |
| California | Public/Mental Health | New Mexico | Child Welfare |
| Colorado | Public/Mental Health | New York | Public/Mental Health |
| Connecticut | Public/Mental Health | North Carolina | Public/Mental Health |
| Delaware | No Definition | North Dakota | Child Welfare |
| Florida | Child Welfare | Ohio | No Definition |
| Georgia | No Definition | Oklahoma | No Definition |
| Hawaii | Public/Mental Health | Oregon | No Definition |
| Idaho | No Definition | Pennsylvania | Child Welfare |
| Illinois | Child Welfare | Rhode Island | Child Welfare |
| Indiana | Child Welfare | South Carolina | No Definition |
| Iowa | Child Welfare | South Dakota | No Definition |
| Kansas | Child Welfare | Tennessee | Public/Mental Health |
| Kentucky | Child Welfare | Texas | Child Welfare |
| Louisiana | No Definition | Utah | Child Welfare |
| Maine | Public/Mental Health | Vermont | No Definition |
| Maryland | No Definition | Virginia | Child Welfare |
| Massachusetts | No Definition | Washington, D.C. | Child Welfare |
| Michigan | No Definition | Washington | No Definition |
| Minnesota | Child Welfare | West Virginia | No Definition |
| Mississippi | No Definition | Wisconsin | No Definition |
| Missouri | Public/Mental Health | Wyoming | No Definition |
| Montana | Public/Mental Health | | |

## Notes

1. California child welfare agency, request for information on “high acuity” definition, 13 March 2024. Respondent identity withheld.
2. Kentucky child welfare agency, request for information on “high acuity” definition, 18 March 2024. Respondent identity withheld.
3. Missouri child welfare agency, request for information on “high acuity” definition, 25 March 2024. Respondent identity withheld.
4. Oregon child welfare agency, request for information on “high acuity” definition, 15 March 2024. Respondent identities withheld.
5. Rhode Island child welfare agency, request for information on “high acuity” definition, 11 March 2024. Respondent identity withheld.
6. Utah child welfare agency, high acuity definitions, 14 March 2024. Respondent identities withheld.
7. Virginia child welfare agency, request for information on “high acuity” definition, 15 March 2024. Respondent identity withheld.
8. District of Columbia Child and Family Services Agency, *FY 2018 needs assessment and FY 2020 resource development plan*, 1 October 2018.
9. Washington child welfare agency, request for information on “high acuity” definition, 12 March 2024. Respondent identities withheld.

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Louisiana Department of Children & Family Services. https://www.dcfs.louisiana.gov/

Maryland Department of Human Services. https://dhs.maryland.gov/

Massachusetts Department of Children & Families. https://www.mass.gov/orgs/massachusetts-department-of-children-families

Mississippi Department of Human Services. https://www.mdhs.ms.gov/

Montana Child and Family Services Division. https://dphhs.mt.gov/cfsd/index

Nebraska Department of Health and Human Services. https://dhhs.ne.gov/

Nevada Division of Child & Family Services. https://dcfs.nv.gov/

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New Mexico Children, Youth, & Families Department. https://www.cyfd.nm.gov/

New York State Office of Children and Family Services. https://ocfs.ny.gov/main/

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